

Aortic Coarctation

ORG: S-2150 (HC)

[Link to Codes](#)

MCG Health

Home Care

28th Edition

This MCG guideline has not yet been updated to reflect new guidance and requirements found in the 2024 Medicare Advantage and Part D Final Rule (CMS-4201-F). For Medicare Advantage beneficiaries, before referring to MCG's guidelines users should first follow all guidance related to this topic located in CMS National Coverage Determinations (NCDs), Local Coverage Determinations (LCDs), and other statutes and regulations. Content used in MCG guidelines may be used to supplement but does not replace, modify, or supersede existing Medicare regulations or applicable NCDs or LCDs.

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Clinical Indications for Admission to Home Healthcare

- Home healthcare is/was needed for appropriate care of the patient because of **ALL** of the following[A][B](1)(2)(3):
 - The patient has had a face-to-face encounter with a physician or allowed non-physician practitioner (NPP) no more than 90 days prior to (or within 30 days after) the start of care related to the home health service needs.[C](4)
 - The patient is Homebound or homebound waiver applies (eg, accountable care organization).(5)(6)
 - The patient's needs can be safely met with Intermittent home care.
 - The patient needs education on condition and self-management care with teach-back.(7)
 - Skilled care at home is required for **1 or more** of the following(8):
 - Cardiovascular status assessment(9)(10)
 - Developmental status assessment[D](11)(12)(13)(14)
 - Home safety assessment(15)(16)
 - Infusion (IV) therapy management (eg, insertion site use and care, infusion administration, monitoring reaction)(17)
 - Medication management, adherence instruction, and side effects assessment(18)(19)
 - Nutrition and hydration management(20)(21)
 - Oxygen or respiratory equipment management (22)(23)
 - Pain management(20)(24)
 - Psychosocial assessment, management, and referrals(25)
 - Respiratory status assessment(26)
 - Significant chronic condition exacerbation with need for clinical intervention and monitoring(10)(27)
 - Wound or dressing management(20)(24)

Visit Data

Home Care Use for Condition

Home care use for condition refers to the frequency with which home care services are provided after inpatient hospitalization for this diagnosis or procedure: Rare: less than 5%; Occasional: from 5% up to 20%; Frequent: from 20% up to 40%; Very Frequent: 40% or greater.

Description	Commercial	Medicare
Home care use for this condition	Occasional	Frequent

Registered Nurse Visit Goal Table

Registered nurse visits are displayed as percentiles that are based on the observed utilization of registered nurse visits for this diagnosis. Individual patients may require fewer or more visits as appropriate for their clinical status and care needs. This table is designed to allow organizations to define their goals for registered nurse utilization by choosing from the displayed range.

Commercial RN Visits					Medicare RN Visits				
10%ile	20%ile	30%ile	40%ile	50%ile	10%ile	20%ile	30%ile	40%ile	50%ile
2	3	3	4	5	2	3	4	5	5

Evaluation and Treatment

General Treatment Course

Stage	Clinical Status	Interventions
1	- Clinical indications for admission met - Home care comprehensive assessment and initial home evaluation complete^[E] QM(28)(29) - The comprehensive assessment should be completed within 5 days after the start-of-care date. - If skilled nursing is ordered, nursing should complete start of care and confirm eligibility. - A qualified rehabilitation therapist (ie, PT, OT, SLP) may open and case manage home care if the orders are for therapy only (if any nursing care is ordered, the case is not considered therapy only). However, Medicare benefits do not cover cases if only occupational therapy (OT) is ordered. - Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment  SR complete for evaluation of current level of functioning (eg,  Activities of Daily Living (ADL) Scoring Tool Calculator) - Psychosocial Assessment  SR complete QM(30) - Social determinants of health screening complete. See Social Determinants of Health Screening Tool  SR for more information. QM - Medication reconciliation complete. See Medication Reconciliation Tool  SR for more information. QM - Home safety assessment complete. See Home Safety Assessment  SR for more information. - Hospital readmission risk factor evaluation QM ^[F](31) - Skilled healthcare needs identified	 Establish and implement interdisciplinary care plan; clinical goals may include QM (32)(33): - Cardiac status improved or stable - Home safety improved or stable - Infection risk reduced^[G] QM(34) - Knowledge deficit improved or resolved - Laboratory values improved or within normal limits - Medication regimen established - Nutrition and hydration status improved or stable - Pain improved - Patient care management improved or stable - Psychosocial issues improved or resolved - Respiratory status improved or stable QM - Vital signs stable - Wound healing without complications - Initiate transition of care planning. QM  Provide treatments and monitoring QM (29): - Cardiac care(35) - Chronic condition management(36) - Compression stocking management(37) - Emergency plan management

- Fall risk management **QM** (38)
- Infusion (IV) therapy management (eg, fluids, antibiotics, parenteral nutrition)(39)
- Laboratory testing or monitoring, if ordered
- Medication management **QM**
- Nutrition management(40)
- Oxygen management **QM**
- Pain management[H] **QM**
- Psychosocial issue and risk management **QM** (19)
- Respiratory care
- Wound management
- Report complications to treating provider, if indicated.
- Coordinate follow-up with treating provider(s).
- Provide patient and caregiver education, reinforcement, and teach-back. **QM** (42)
 - Review patient management techniques (eg, medication, activity, monitoring).
 - Review complications and when and to whom to report.
 - See corresponding Patient Education for Clinicians for more information.
- ☐ Initiate referrals as indicated. (43)
 - Social worker referral
 - Home Health Aide (HHA) referral
 - Educational program (eg, chronic condition management, self-management)(44)
 - Financial, for follow-up care, medication, and transportation(19)
 - Self-management information(45)
 - Tobacco use treatment, as needed **QM** (46)(47)(48)
 - Other

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- Therapeutic response to interventions **QM**
- Patient/caregiver able to monitor and self-manage condition, or unwilling or unable to be trained after sufficient efforts made (49)
- Interdisciplinary care plan continues
- Ongoing assessment of clinical needs
- ☐ Provide treatments and monitoring **QM** :
 - Cardiac care
 - Chronic condition management

- Compression stocking management
- Emergency plan management
- Fall risk management **QM**
- Infusion (IV) therapy management (eg, fluids, antibiotics, parenteral nutrition)
- Laboratory testing or monitoring, if ordered
- Medication management **QM**
- Nutrition management
- Oxygen management **QM**
- Pain management **QM**
- Psychosocial issue and risk management **QM**
- Respiratory care
- Wound management
- Continue patient and caregiver education, reinforcement, and teach-back. **QM**
 - Assess patient management ability and techniques.
 - Provide instructional support as needed.
 - Verify understanding of complications and when and to whom to report.
- Report complications to treating provider, if indicated.
- Continue transition of care planning. **QM**
- Verify follow-up with treating provider.
- Follow up on referral status, as indicated.

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- **IV therapy discontinued or can be managed independently**
 - **Inserted or implanted device discontinued, or functioning normally and can be managed independently(17)**
 - Medical equipment and supplies available and safe use demonstrated(15)
 - **Medical status stable for patient's condition(29)**
 - **Medication regimen established and reconciliation completed QM**
 - **Patient/caregiver able to monitor and self-manage condition independently, or unwilling or unable to be trained after sufficient efforts made(49)**
 - **Patient Safe at home or appropriate referrals made QM(50)**
 - Psychosocial issues adequately addressed or appropriate referrals made **QM**
 - **Significant chronic condition exacerbation resolved**
 - **Transition plans understood**
 - **Wound(s) healing without complications, or complications resolving**

- **Transition of care planning completed QM**
- Discharge from home care services

Recovery Milestones are indicated in **bold**.

Extended Visits

Additional skilled visits may be: brief (1 visit), moderate (2 to 3 visits), or prolonged (more than 4 visits).

- Additional nursing visits may be needed for **1 or more** of the following:

- ☐ Failure to meet discharge criteria (Recovery Milestones within final stage of General Treatment Course)
 - Expect brief visit extension
- ☐ Initiate Home Safety Assessment [SR](#) due to change in patient's ability to perform activities of daily living (ADL) or instrumental activities of daily living (IADL).(15)
 - Expect brief visit extension.
- ☐ Infusion therapy administration and assessment; examples include(17)(51):
 - New intravenous medication until safe tolerance established
 - Complications of infusion therapy (eg, extravasation of vesicant)
 - Central venous access device (CVAD) sterile dressing change unmanageable by patient or in outpatient setting
 - Peripheral IV access initiation or site rotation
 - Administration of agent requiring on-site monitoring by clinical personnel
 - Transition from parenteral to enteral nutrition or oral feeding, or from enteral nutrition to oral feeding
 - Infusion therapy unmanageable by patient or caregiver while intense and complex care needs make home care safer and more practical than outpatient infusions
 - Expect brief to moderate visit extension.
- ☐ Ongoing education required for patient or caregiver as indicated by demonstrated ability, willingness to learn, and(7)(29):
 - Need for improved understanding of patient's medical condition, diagnosis, disease, or disability
 - Need for improved understanding of methods and means to manage aspects of medical condition
 - Need for interventions to increase adherence to therapy
 - Expect brief to moderate visit extension.
- ☐ Pain evaluation and management; examples include(33)(52)(53):
 - Frequent pain assessment using validated diagnostic tool[]
 - Early referral and treatment of pain
 - Ongoing education and training of pain treatment plan (eg, multimodal pain management)[]
 - Pain medication side effects or risk factors for adverse effects (eg, constipation, sedation, falls)
 - Expect brief to moderate visit extension.
- ☐ Respiratory evaluation and equipment management; examples include(54):
 - Breathing abnormalities and oxygen regimen not established
 - Laboratory values outside normal limits; examples include:
 - Oxygen saturation below 90% (or below baseline)
 - Cultures positive or identification of infective source pending, and treatment regimen not established
 - Respiratory status assessment appropriate when necessary to evaluate patient's response; examples include(22):
 - Change in oxygen settings or oxygen discontinued at home
 - Periodic (eg, every 3 months) evaluation of oxygenation status to evaluate effectiveness(55)
 - Change in continuous positive airway pressure (CPAP) or bilevel positive airway pressure (BPAP) settings
 - Possible need for humidifier
 - Ongoing education and training on respiratory management program and oxygen delivery equipment needed
 - Expect moderate to prolonged visit extension.
- ☐ Significant chronic disease exacerbation, with need for skilled clinical intervention and monitoring(29)
 - Expect brief visit extension.
- ☐ Wound management; examples include(24)(56):
 - Wound larger, or change in appearance (eg, appearance of eschar or new purulent drainage) since last assessment
 - Wound status change, with new type of dressing or procedure
 - New wound development (eg, pressure injury)
 - Topical negative pressure dressing change, or pump malfunction, if used
 - Expect brief to moderate visit extension.

Patient Education

- Patient and caregiver education. See:
 - Aortic Coarctation, Angioplasty, Adult: Patient Education for Clinicians [SR](#)
 - Aortic Coarctation, Angioplasty, Pediatric: Patient Education for Clinicians [SR](#)
 - Aortic Coarctation, Excision or Repair, Adult: Patient Education for Clinicians [SR](#)
 - Aortic Coarctation, Excision or Repair, Pediatric: Patient Education for Clinicians [SR](#)

Discharge Planning

- Transition of care planning may include(29)(50):
 - ☐ Assessment and early establishment of anticipated discharge destination and plans for care, including:
 - Treatment plan development involving multiple providers
 - Premorbid functioning(57)

- Patient and caregiver preferences and abilities
- Psychosocial status. See Psychosocial Assessment [SR](#) for further information. **QM**
- Social determinants of health screening. See Social Determinants of Health Screening Tool [SR](#) for further information. **QM(58)**
- Acceptance for care for patient at next level of care, as appropriate
- Transition of care plan complete, which may include:
 - Patient and caregiver education complete. See Teach Back Tool [SR](#) for further information.(7)
- ☐ Medication reconciliation completion includes **QM** (59)(60):
 - Compare patient's discharge list of medications (prescribed and over-the-counter) against provider's admission or transfer orders.
 - Assess each medication for correlation to disease state or medical condition.
 - Report medication discrepancies to prescribing provider, attending physician, and primary care provider, and ensure accurate medication order is identified.
 - Provide reconciled medication list to all treating providers.
 - Confirm that patient or caregiver can acquire medication.
 - Educate patient and caregiver.
 - Provide complete medication list to patient and caregiver.
 - Importance of presenting personal medication list to all providers at each care transition, including all provider appointments
 - Reason, dosage, and timing of medication (eg, use "teach-back" techniques)(18)
 - Encourage communication between patient, caregiver, and pharmacy for obtaining prescriptions, setting up home medication delivery, and reviewing for drug-drug interactions.
 - See Medication Reconciliation Tool [SR](#) for more information.
- ☐ Appointments planned or scheduled:
 - Primary care provider
 - Cardiologist(61)
 - Outpatient imaging, tests, or procedures(10)(19)(62)
 - Specialists for management of comorbidities as needed(61)
 - Surgeon(61)
 - Other
- Assistance and support referrals completed
- ☐ Medical equipment and supplies coordinated (ie, delivered or delivery confirmed), as indicated:
 - Cardiac care equipment and supplies (eg, compression stockings)(63)
 - Medication equipment and supplies
 - Respiratory equipment and supplies
 - Wound care equipment and supplies
 - Other
- Transition plan communicated to all members of patient's care team **QM**

Quality Measures

- HEDIS Measures include(64):
 - Medication reconciliation is completed.
 - Transition of care needs are assessed and addressed.
 - Psychosocial status (eg, depression screening) is assessed and addressed.
 - Fall risk is assessed and addressed.
 - Tobacco use is assessed and addressed.
 - Immunization status is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Social needs screening and intervention is completed.
- Home Health Compare Quality Measures include(65):
 - Readmission risk is assessed and addressed.
 - Oral medication management is assessed for improvement.
 - Timely initiation of care is assessed and addressed.
 - Medication reconciliation and education are completed.
 - Immunization status is assessed and addressed.
 - Dyspnea is improved.
 - Discharge to community is completed.
- Home Health Quality Reporting Program quality measures include(66):
 - Immunization status is assessed and addressed.
 - Medication reconciliation and education are completed.

- Readmission risk is assessed and addressed.
 - Fall risk is routinely assessed.
 - Pain status is assessed and pain is addressed.
 - Oral medication management is assessed for improvement.
 - Timely initiation of care is assessed and addressed.
 - Transfer of health information to provider and patient is completed.
 - Dyspnea is improved.
- The Joint Commission Home Care National Patient Safety Goals include(67):
 - Medication reconciliation is completed. Education is provided on medication administration including the importance of bringing up-to-date medication lists to all provider visits.
 - Healthcare-associated infection risk is reduced.
 - Fall risk is assessed and addressed.
 - Home oxygen therapy safety risk is assessed.

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Footnotes

[A] **Evidence:** A retrospective cohort study based on 1151 Medicare accountable care organization (ACO) patients suggests utilizing a transitional care planning approach to prioritize discharging patients to the least restrictive next level of care, as patients discharged to skilled nursing facilities were almost 5 times more likely to be readmitted to the hospital at 30 days compared to patients discharged to the home healthcare setting.(1) [A in Context Link 1]

[B] **Evidence:** A retrospective cohort study of 25,357 older patients discharged from a skilled nursing facility (SNF) found that patients who received services from a Medicare-certified home health agency had decreased odds of rehospitalization or SNF readmission as well as lower mortality risk.(2) [B in Context Link 1]

[C] **National guidelines:** The Centers for Medicare and Medicaid Services (CMS) indicates that a face-to-face encounter must occur no more than 90 days prior to the home health start-of-care date or within 30 days of the start of the home healthcare as related to the primary reason the patient requires home health services. Supporting documentation requirements include the allowed provider type, the date of encounter, the need for skilled services, the patient's homebound status, and that the encounter was related to the primary reason the patient requires home health services.(4) [C in Context Link 1]

[D] **Specialty society:** American Academy of Pediatrics recommends ongoing psychosocial surveillance with the use of standardized developmental screening tests to identify cognitive, emotional, and behavioral concerns from birth through age 5 years.(11)(12) [D in Context Link 1]

[E] **National guidelines:** The Centers for Medicare and Medicaid Services uses a data collection system to establish best practices, understand costs, and identify cost-effective systems and procedures. The current data collection system for eligible patients is the Outcome and Assessment Information Set (OASIS). OASIS is an assessment tool used to identify patient problems, strengths, and preferences. The OASIS used to create an individualized care plan can be found by going to www.cms.gov and searching OASIS Data Sets.(28) [E in Context Link 1]

[F] **Evidence:** A systematic review of 28 studies based on older cohorts of home healthcare patients found that skin ulcers, psychiatric conditions, dyspnea/COPD, cardiovascular conditions, diabetes, functional deficits, more comorbidities, and higher medication usage were associated with higher risk of hospital readmissions.(31) [F in Context Link 1]

[G] Infection prevention and control measures are an important part of infectious disease management that should be maintained by all clinicians as well as an important component of the patient's self-management education. Hand hygiene, usage of personal protective equipment (PPE), and managing environment and clinical equipment are some cross-infection prevention techniques that should be maintained with every visit per organization protocol.(34) [G in Context Link 1]

[H] Naloxone rescue training and education about overdoses should be offered to the patient and family/caregiver as indicated.(41) [H in Context Link 1]

[I] Pain assessment should include how pain affects patient's physical and psychosocial function and progress toward treatment goals.(52) [I in Context Link 1]

[J] The patient's agreed-upon written treatment plan should include measurable and realistic functional or quality-of-life goals, pain-management education, treatment options, and safe use of opioid and nonopioid medications as prescribed.(41)(52) [J in Context Link 1]

[K] **National guidelines:** The Centers for Disease Control and Prevention (CDC) recommends nonpharmacologic and nonopioid therapy before opioids for management of chronic pain (not due to malignancy). Considerations for opioids should include pain-management goal setting and determining if benefits outweigh risks. When opioids are prescribed, they should be provided in conjunction with nonpharmacologic pain-management interventions and planned monitoring for misuse or use escalation, including naloxone education.(41) [K in Context Link 1]

Definitions

Custodial care

- Custodial care is personal unskilled care to support the patient's care of activities of daily living (ADL), such as bathing, dressing, and eating.(1)

References

1. Mauk KL. Post-acute care. In: Larsen PD, editor. Lubkin's Chronic Illness: Impact and Intervention. 11th ed. Jones & Bartlett Learning; 2022:477-520.

Home Health Aide (HHA) referral

- Home Health Aide (HHA) referral may be considered when **ALL** of the following exist(1)(2):
 - HHA supports skilled treatment plan (ie, facilitates treatment, prevents deterioration).
 - Nonskilled service needed for hands-on personal care (eg, bathing or dressing assistance), or other service to maintain patient's health (eg, repetitive practice exercises) or facilitate treatment (eg, simple dressing changes not requiring services of licensed nurse)
 - Significant functional limitations and:
 - Patient cannot perform service themselves.
 - There is no able and willing caregiver available to support patient care needs, or there is an able and willing caregiver available, but patient will not allow services to be performed by that caregiver.
 - Absence of functional plateau

References

1. Centers for Medicare and Medicaid Services. "Dependent services requirements." 42 CFR Pt. 409.45 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
2. Centers for Medicare and Medicaid Services. "Condition of participation: Home health aide services." 42 CFR Pt. 484.80 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Homebound

- Patients are homebound if they have functional impairments that make leaving home a considerable taxing effort or are advised by their provider not to leave home. Homebound status may not be required if there are circumstances important for home monitoring of

the patient's condition (eg, patient may need clinical evaluation of home regimen of chronic disease). Pediatric patients may have different considerations for homebound status, including infection prevention, dyspnea during activity, or home confinement for medical issues.[A](1)(2)(3)

References

1. Home Health Services. Chapter 7 Rev 11447 In: Medicare Benefit Policy Manual [Internet-Only Manual] [Internet] Centers for Medicare and Medicaid Services. 2022 Jun Accessed at: <https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Internet-Only-Manuals-IOMs-Items/CMS012673>. [accessed 2023 Sep 21]
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3. Wright EM, Sharps P, Flagg J, Busch D. Health planning for maternal-infant and child health settings. In: Savage CL, editor. Public / Community Health and Nursing Practice: Caring for Populations. 2nd ed. Philadelphia, PA: F.A. Davis; 2020:420-446.

Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services indicates that a considerable taxing effort generally includes the use of supportive devices such as crutches, canes, wheelchairs, and walkers; the use of special transportation; or the assistance of another person to leave home. A patient whose absences are infrequent, for short duration (eg, religious services, graduations), or necessary for healthcare (eg, dialysis, outpatient chemotherapy, medical day care) may still be considered homebound.(1)

Intermittent home care

- Intermittent home care is defined as part-time services that are provided in the home.[A](1)(2)(3)

References

1. Home Health Services. Chapter 7 Rev 11447 In: Medicare Benefit Policy Manual [Internet-Only Manual] [Internet] Centers for Medicare and Medicaid Services. 2022 Jun Accessed at: <https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Internet-Only-Manuals-IOMs-Items/CMS012673>. [accessed 2023 Sep 21]
2. Deutsch A. Health policy. In: Larsen PD, editor. Lubkin's Chronic Illness: Impact and Intervention. 11th ed. Jones & Bartlett Learning; 2022:569-584.
3. Community-based services. In: Chies S, editor. Pratt's Long-Term Care. 5th ed. Jones & Bartlett Learning; 2022:183-218.

Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services defines intermittent care as services that must be less than 8 hours each day and 28 or fewer hours each week (or, for special cases, up to 35 hours each week).(1)

Oxygen or respiratory equipment management

- **Specialty society:** Medical guidelines recommend that patients getting home oxygen therapy be evaluated by a nurse or licensed clinician with experience in home oxygen use within 4 weeks of home therapy initiation.(1)(2)(3)

References

1. Hardinge M, et al. British Thoracic Society guidelines for home oxygen use in adults. Thorax 2015;70 Suppl 1:i1-43. DOI: 10.1136/thoraxjnl-2015-206865. (Reaffirmed 2023 Jan)
2. Jacobs SS, et al. Home oxygen therapy for adults with chronic lung disease. an official American Thoracic Society clinical practice guideline. American Journal of Respiratory and Critical Care Medicine 2020;202(10):e121-e141. DOI: 10.1164/rccm.202009-3608ST. (Reaffirmed 2023 Jan)
3. Hayes D, et al. Home oxygen therapy for children. an official American Thoracic Society clinical practice guideline. American Journal of Respiratory and Critical Care Medicine 2019;199(3):e5-e23. DOI: 10.1164/rccm.201812-2276ST. (Reaffirmed 2023 Jan)

Patient/caregiver able to monitor and self-manage condition, or unwilling or unable to be trained after sufficient efforts made

- **National Guideline:** The Centers for Medicare and Medicaid Services indicates that continued teaching and training is not reasonable if the patient and/or caregiver cannot or will not be trained in an appropriate time frame. In these situations, considerations for an alternative level of care and referrals to community resources are appropriate to meet patient needs.(1)

References

1. Centers for Medicare and Medicaid Services. "Beneficiary qualifications for coverage of services." 42 CFR Pt. 409.42 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Safe at home

- Home safety may be determined by a completed Home Safety Assessment and the patient being in no imminent harm. Harm includes high risk for deterioration or injury and may include risk of harm to self or others.(1)(2)

References

1. Laplante N. Home care safety. In: Perry AG, Potter PA, Ostendorf WR, editors. Nursing Interventions and Clinical Skills. 7th ed. Elsevier; 2020:842-862.
2. Ekroos RA, Traveller L. Violence and abuse. In: Rector C, Stanley MJ, editors. Community and Public Health Nursing Promoting the Public's Health. 10th ed. Wolters Kluwer; 2022:506-536.

Skilled care

- Skilled care is that which must be performed or supervised by professional or technical personnel, be reasonable and necessary for patient's treatment, and exceed scope of Custodial care. Skilled care may be direct (eg, in-person assessment and administration of treatments) or indirect (eg, coordinating care between providers, supervising care aides).[A](1)(2)(3)(4)

References

1. Centers for Medicare and Medicaid Services. "Criteria for skilled services and the need for skilled service." 42 CFR Pt. 409.32 Washington, DC 2023 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
2. Harding MM. Professional nursing. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:1-18.
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4. Centers for Medicare and Medicaid Services. Medicare Program Integrity Manual. Chapter 6 - Medical Review of Inpatient Hospital Claims for Part A Payment Rev. 10365 [Internet] Centers for Medicare and Medicaid Services. 2020 Oct 02 Accessed at: <https://www.cms.gov/regulations-and-guidance/regulations-and-guidance>. [accessed 2023 Aug 31]

Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services defines skilled care as a service that is "so inherently complex that it can only be safely and effectively performed by, or under the supervision of, professional or technical personnel."(1)

Social worker referral

- Social worker (SW) referral may be considered when **1 or more** of the following exist[A](3)(4):
 - Social or emotional problems that impede medical treatment or recovery(5)
 - Abuse or neglect suspected
 - Advance care planning (eg, end-of-life care, advance directives)
 - Financial or legal assistance, transportation, or housing referrals needed
 - Decision-making facilitation needed within family or among caregivers

References

1. Gandy J. Legal and ethical basis for practice. In: Fosbre CD, Varcarolis EM, Chiappetta L, editors. Varcarolis' Essentials of Psychiatric-Mental Health Nursing. 5th ed. Elsevier; 2023:60-73.
2. Barker KDD, Johnson MM. The psychiatric history and physical examination. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:1-41.e2.
3. de Wilde C. Social work. In: O'Hanlon S, Smith M, editors. Comprehensive Guide to Rehabilitation of the Older Patient. 4th ed. Elsevier; 2021:91-94.
4. Harding MM. Professional nursing. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:1-18.
5. Centers for Medicare and Medicaid Services. "Dependent services requirements." 42 CFR Pt. 409.45 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Footnotes

- A. Social worker (SW) subspecialties include medical, mental health, and substance use. The type of SW referral should include consideration of subspecialty, diagnosis of the patient, and the purpose for referral.(1)(2)

Transition of care planning completed

- Transition of care planning completed, including **ALL** of the following(1)(2):

- Transition plan communicated to all members of patient's healthcare team
- Summary of care, discharge list of medications, and transition plan communicated to primary care provider
- Medication reconciliation complete
- Education on condition management and complications to report provided to patient or caregiver[A]
- Follow-up appointments scheduled
- Referrals to continue medical and rehabilitation goals (eg, outpatient) arranged, if needed
- Services (eg, equipment, environment modifications, transportation) arranged, if needed
- Psychosocial issues addressed, or plan for management, if needed

References

1. Saleeby J. Communication and collaboration. In: Perry AG, Potter PA, Ostendorf WR, editors. Nursing Interventions and Clinical Skills. 7th ed. Elsevier; 2020:9-21.
2. Transitions of care. In: Gillingham DC, editor. Foundations of Case Management. Blue Bayou Press; 2021:137-144.
3. Hudson T. The role of social determinates of health in discharge practices. Nursing Clinics of North America 2021;56(3):369-378. DOI: 10.1016/j.cnur.2021.04.004.

Footnotes

- A. Health education materials should be given in patient's and caregiver's native language using trained language interpreters whenever possible.(3)

Transition plans understood

- Transition plans and education understood, including(1)(2):
 - Patient and caregiver show ability and understanding to manage condition and treatments.
 - Patient and caregiver know signs and symptoms of complications, whom to contact, and how and when to seek medical intervention.

References

1. Morris BC. Patient and caregiver teaching. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:49-62.
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Codes

ICD-10 Diagnosis: Q25.1, Q25.21, Q25.29, Q25.3, Q25.42, Z45.1, Z45.2, Z48.01, Z48.812

ICD-10 Procedure: 027W3DZ, 027W3ZZ, 02BW0ZZ, 02BX0ZZ, 02BX3ZX, 02BX3ZZ, 02RW07Z, 02RW08Z, 02RW0JZ, 02RW0KZ

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